



Infection Prevention and Management Manual

Single Room Prioritisation and Cohorting Guideline

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff, Medical, Nursing, Allied Health, Pharmacy, Facilities Management, Clinical Corporate staff.

Introduction

This document guides the allocation of single patient rooms and cohorting of patients requiring isolation to optimise bed usage, patient flow and to risk-manage infection transmission within inpatient areas of SCGOPHCG.

Priority for single room allocation will always be given to those patients who are required to isolate from other patients.

All decisions to cohort patients for any infection control reason must be in accordance with [Infection Prevention and Management](#) policy.

Single Room allocation

Single room allocation should be prioritised in the following order:

1. Patients being managed under airborne precautions (negative pressure isolation room [NPIR]) e.g. TB, measles, chickenpox and disseminated shingles, Ebola
2. Patients being managed under modified airborne precautions: COVID-19 positive patients
3. Patients being managed under droplet precautions
4. Patients being managed under contact precautions
5. Patients being managed under contact precautions as multi-resistant organism (MRO) contacts
 - a) Micro-Alert H and K patients
 - b) Micro-Alert F and W patients
6. Palliative patients imminently dying or Voluntary Assisted Dying (VAD) patients
7. Neutropenic patients (Neutrophils ≤ 0.5) or patients less than 3 months post organ transplant
8. Patients with unsociable behaviours

Cohorting

Cohorting patients with like diseases/infections must be used as a strategy when no single rooms are available or to preserve single room numbers in the face of significant common infections e.g. seasonal influenza or COVID-19.

Infections that can be cohorted:

- Respiratory viruses, confirmed by PCR, **AND** with no other Micro alerts or infection requiring contact precautions. This includes COVID-19, influenza, parainfluenza, respiratory syncytial virus and human metapneumovirus, rhinovirus, adenovirus, and seasonal coronaviruses. Only patients with the same single respiratory virus may be cohorted together **AND** where the patient is not excluded from cohorting as outlined below.
- Patients with diarrhoea due to the same infection confirmed by PCR, e.g. *C. difficile*, norovirus, rotavirus **AND** where the patient is not excluded from cohorting as outlined below.
- Patients with dermatomal shingles confirmed by PCR **AND** where the patient is not excluded from cohorting as outlined below.
- Micro-Alert B and V patients are suitable for cohorting with patients of the same single Micro-Alert where the patient has not otherwise been excluded from cohorting as outlined below.
- Patients with other infections as directed by Infection Prevention and Management (IPM) **IF** the patient is not excluded from cohorting as outlined below.

Who cannot be cohorted:

- Patients requiring airborne precautions for TB, measles, chickenpox and disseminated shingles, Ebola. These cases should be prioritised to a NPIR.
- Patients who are Micro-Alert C, G and J are not suitable for cohorting unless directed by IPM.
- Patients who are MRO contacts (Micro-Alert H, K, F and W) unless directed by IPM.
- Patients with diarrhoea of unknown cause should remain isolated in a single room with ensuite.
- Patients hospitalised outside of Western Australia in the last 12 months should remain isolated in a single room under contact precautions until clearance screens complete and results finalised.
- High-risk patient e.g. transplant and/or haematology, cystic fibrosis and severely immunocompromised.

Principles

- Cohorting of like infections will be initiated as a response to bed management and patient flow requirements.
- Patient flow staff will coordinate cohorting spaces.
- Patients will be transferred from their natural home ward to a cohort room if suitable to transfer to optimise bed usage. Medical teams will continue to manage these patients as an outlier to their home ward.
- Consider the bariatric patient and patients with unique equipment needs where in room use of equipment may compromise patient and staff safety.

This guideline should be considered in conjunction with the following policy and Guidelines:

- [SCGOPHCG Hospital Capacity and Demand Management Framework](#)
- [Specialty Area Admission Criteria](#)
- [SCGOPHCG Bed Configuration](#)
- [SCGOPHCG Infection Prevention and Management Manual](#)
- [COVID-19 Policy](#)
- [Multi-Resistant Organism Policy](#)
- [Influenza-like illness policy](#)

Authorisation & Version Control

Policy Sponsor	SCGOPHCG Director of Cancer, Imaging and Clinical Services				
Policy Contact	SCGOPHCG Coordinator of Nursing Infection Prevention and Management				
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Standards Applicable	Std 1: Clinical Governance  Std 2: Partnering with Consumers  Std 3: Preventing and Controlling Healthcare Associated Infection 				

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

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